

GENERATIVE AI FOR PRIOR AUTHORIZATION, DENIALS AND APPEALS IN HEALTHCARE

MODULE 1 REVISED INSTRUCTOR LED VERSION

Prior Authorization Foundations, Failures and Public PA Resource Workflow Using AMA and CMS

Revision marker: Text shown in red identifies the areas changed from the earlier CoverMyMeds version to the AMA and CMS public resource version.

For PA Coordinators, Revenue Cycle Professionals, Physicians, Billers, Administrators and Instructional Design Teams

Document Purpose

This revised version keeps the approved instructional structure of Module 1. The only major change is the replacement of CoverMyMeds because the tool may not reopen reliably in India and may create access issues during learner registration.

The replacement tools are not commercial prior authorization submission platforms. They are public reference tools. This distinction matters. Module 1 is foundational, so the learner needs to understand prior authorization concepts, workflow risks, documentation failure points and payer language before learning submission tools in later modules.

The two tools used in this revised module are the AMA Prior Authorization resources and CMS Prior Authorization resources. Both are free public websites, accessible without normal account registration, and useful for international teaching because learners can open them as educational references from most locations. They help explain what prior authorization is, why it creates administrative burden, how electronic prior authorization is evolving and why documentation and workflow control matter.

Course Overview and Module Positioning

This professional course is designed for people who work around prior authorization, medication access, denials, appeals, billing and revenue cycle operations. The course is not written as a technical software manual. It is written as an instructor led professional learning experience where the learner understands the healthcare problem first and then learns how public resources can support safe workflow thinking.

Module 1 is deliberately foundational. A learner cannot write strong appeals or manage denials well without first understanding prior authorization. Most workflow failures happen before the denial letter is ever received. A missing PA check at scheduling, a vague chart note, an incomplete step therapy history, or an untracked payer request can become a denial days or weeks later. This module helps learners slow down and see the full system.

The tools used in this module are AMA Prior Authorization resources and CMS Prior Authorization resources. They are selected because they are free, public, internationally accessible reference resources and do not require learners to register for a country restricted platform. They help learners understand prior authorization burden, reform language, electronic PA direction, documentation expectations and policy concepts. They do not submit PA requests, guarantee approval or replace payer portals.

Module	Title	Recommended Duration	Level	Tools Used
1	Prior Authorization Foundations, Failures	90 to 120 minutes	Foundational	AMA Prior Authorization resources and CMS

	and Public PA Resource Workflow			Prior Authorization resources
2	Clinical Documentation That Gets Approved	60 to 90 minutes	Intermediate	To be selected later
3	Decoding Denials and Root Cause Analysis	60 to 90 minutes	Intermediate to Advanced	To be selected later
4	Appeals, Peer to Peer Review and Winning Letters	60 to 90 minutes	Advanced	To be selected later
5	Building Scalable PA and Denial Workflows	60 to 90 minutes	Operational	To be selected later
6	HIPAA, Regulation, Ethics and Safe AI Use	60 to 90 minutes	Regulatory	Compliance and internal governance tools

Micro Module	Topic	Teaching Depth	Core Output
1.1	What prior authorization means in real healthcare practice	Full concept lesson with examples and role play	Learner can explain PA, medical necessity and payer approval
1.2	The people involved in prior authorization and why handoffs fail	Full workflow lesson with actor responsibilities	Learner can identify role based responsibility and missing documentation points
1.3	Why prior authorization requests fail	Full denial classification lesson with public resource support	Learner can separate administrative failure from clinical documentation failure
1.4	Using AMA and CMS resources in a PA learning workflow	Full tool lesson with free status, history, purpose, competitors, steps and case studies	Learner can use public resources to understand PA concepts, reform direction and workflow risks

Module Introduction

Prior authorization is one of the most important pressure points in healthcare operations because it sits at the intersection of patient access, provider documentation, payer rules, pharmacy access and revenue cycle performance. A provider may make the right clinical decision, but the organization can still lose time, money and patient trust if the prior authorization process is weak. This is why prior authorization must be taught as a workflow, not as a form.

In everyday practice, prior authorization problems rarely appear as one clean issue. A medication may be denied because the payer says step therapy was not completed, but the real problem may be that the provider documented failed prior therapy without naming the drug, dose, start date, stop date, response or adverse effect. An imaging request may be denied because conservative therapy was not documented. A specialty drug may be approved but still fail at the pharmacy because the approval is linked to the wrong pharmacy pathway. These are operational and documentation problems, not just payer problems.

This module teaches the learner to think like a professional prior authorization coordinator. That means asking the right questions before submission. Is prior authorization required? Which payer rule applies? What diagnosis code supports the request? What clinical facts prove medical necessity? What prior treatments must be documented? Which attachments are required? Who must provide missing information? How will the decision be tracked? If approved, what are the approval dates and limits? If denied, what is the next action?

The module uses AMA and CMS public resources because the training problem here is not how to click through a private payer tool. The training problem is how to understand prior authorization language, workflow burden, policy direction and documentation risk. AMA and CMS resources support that goal without forcing learners into a country restricted registration process.

Critical Safety Rule for All Training Activities

Use fictional or deidentified examples only. Do not use real patient names, dates of birth, member IDs, medical record numbers, addresses, claim numbers, screenshots, insurance cards, full chart notes or complete denial letters during training. If a real organization wants to use any tool with patient information, the organization must review privacy terms, security controls, access permissions and whether a business associate agreement is needed.

Tools Used in Module 1

Tool 1: AMA Prior Authorization Resources

The AMA Prior Authorization resources are public educational and advocacy resources created by the American Medical Association. They explain what prior authorization is, why it creates administrative burden, how it affects physicians and patients, and what reform efforts are underway. For this course, the AMA resource is used as a teaching and discussion tool, not as a PA submission tool.

Why this tool is used

This tool is used because it gives learners accessible language for explaining prior authorization burden, patient delay, workflow frustration and the need for reform. It is especially helpful for Micro Modules 1.1, 1.2 and 1.3 because learners must first understand the problem before they learn how to solve it.

Free status and access

The AMA prior authorization page is publicly accessible and does not require normal learner registration to read the core material. Some AMA materials may have member or newsletter options, but the core prior authorization resource page can be used as a free public reference in training.

Brief history and credibility

The American Medical Association is a long established physician organization in the United States. It has published extensive material on physician practice management, prior authorization burden, reform efforts and payer process concerns. For a global learner, the AMA resource is useful because it shows how a major medical organization frames prior authorization as both an administrative and patient access issue.

How it helps solve the learning problem

The AMA resource helps learners understand the real world burden of prior authorization. It gives language for explaining why delays happen, why manual processes are inefficient and why practices need better documentation and workflow discipline. It is not used to answer patient specific payer questions. It is used to build foundational understanding and professional vocabulary.

Tool 2: CMS Prior Authorization Resources

CMS Prior Authorization resources are public resources from the Centers for Medicare and Medicaid Services. CMS resources explain prior authorization, pre claim review, electronic prior authorization concepts and Medicare related policy direction. For this module, CMS is used as a source of public regulatory and operational language.

Why this tool is used

CMS resources are used because learners must understand that prior authorization is not only a practice burden. It is also connected to payer operations, coverage review, documentation support, electronic data exchange and regulatory expectations. CMS gives learners a more formal payer and government perspective.

Free status and access

CMS pages are public government resources and can be opened without normal account registration. They are suitable for international learners because they are reference pages, not account based submission portals.

Brief history and credibility

CMS is the federal agency that administers Medicare, Medicaid and related health programs in the United States. CMS has published resources on prior authorization, electronic prior authorization and pre claim review concepts. For a course on prior authorization, denials and appeals, CMS resources help learners understand official terminology and the direction of digital PA reform.

How it helps solve the learning problem

CMS resources help learners understand definitions, timing differences, electronic prior authorization direction and the importance of supporting documentation. They support safe learning because learners can practice reading official language without entering patient data or creating accounts.

Micro Module 1.1: What Prior Authorization Means in Real Healthcare Practice

Introduction

Prior authorization is often explained as getting approval from the insurance company, but that explanation is too thin for a healthcare professional. Prior authorization is a payer review process where selected services, medications, devices, procedures, imaging studies, therapies or admissions must be reviewed before the payer agrees to cover them under the patient's health plan. The provider may believe the requested care is clinically correct, but the payer still wants proof that the request meets coverage rules.

This difference matters because many new PA coordinators assume that a doctor's order should be enough. In reality, the order is only the starting point. A payer may require diagnosis details, duration of symptoms, prior treatment attempts, medication history, lab values, imaging results, specialist notes, functional limitation, site of care justification or a specific code combination. If those items are missing, the request can be denied even when the treatment may be reasonable.

Detailed Teaching Explanation

Prior authorization has two sides. The first side is clinical need. Clinical need is the provider's judgement that a patient requires a medication, test, procedure or service. The second side is payer coverage approval. Payer approval is the payer's decision that the submitted request meets plan criteria. These two sides must be connected by documentation. Documentation is the bridge between what the provider knows and what the payer can approve.

For example, consider an adult patient with low back pain. The provider may believe a lumbar MRI is needed because the symptoms are worsening. The payer may not approve based only on the phrase worsening pain. The payer may ask whether the pain has lasted long enough, whether conservative treatment was tried, whether there are neurological symptoms, whether red flags are present, whether prior imaging was done and whether the correct CPT and ICD 10 codes were used. If the PA submission does not answer those questions, the payer may deny.

Medication prior authorization works in a similar way. A specialist may prescribe a biologic medication because standard therapy has not worked. The payer may ask whether the patient tried the preferred medication first, what dose was used, how long it was used, what response occurred and why the requested medication is now needed. The phrase failed previous therapy is usually too vague. A payer needs exact treatment history. The PA coordinator needs to learn that vague clinical language must be converted into payer ready evidence.

A strong PA request does not simply say please approve. It tells a clear story. The story begins with diagnosis, then explains severity, prior treatments, outcomes, contraindications, current symptoms, objective evidence and the reason the requested care is appropriate now. A PA request is not creative writing. It is evidence based administrative communication. Every sentence should help the payer reviewer answer one question: does this request meet criteria?

Learners should also understand that prior authorization is not always wrong in concept. Payers use it to control utilization, reduce unnecessary care and apply plan coverage rules. The problem is that the process often becomes

slow, inconsistent and burdensome. A well trained PA professional cannot remove the payer requirement, but they can reduce avoidable failure by submitting complete, accurate and timely information.

How the AMA and CMS Resources Support This Lesson

The AMA resource helps the instructor frame the human and operational burden of prior authorization. It explains how PA requirements can create delays, practice burden and patient access problems. The CMS resource helps explain prior authorization as a formal review concept where supporting documentation is submitted before or around coverage review. Together, the two resources allow learners to see both sides: the physician practice burden and the payer or regulatory process language.

Example of Weak and Strong PA Thinking

Weak thinking: The physician ordered it, so the payer should approve it. This is weak because it ignores payer criteria and documentation burden. The payer reviewer is not present in the exam room. The reviewer sees only what is submitted.

Strong thinking: The physician ordered it. Now we must show the payer why the order meets coverage criteria using diagnosis, history, failed treatment, objective findings, correct codes and supporting records. This is stronger because it accepts the reality of payer review and prepares the request accordingly.

Weak documentation: Patient has knee pain and needs surgery. This does not explain severity, imaging, failed therapy, duration, functional impact or why surgery is necessary now.

Stronger documentation: Adult patient with severe right knee osteoarthritis confirmed by imaging. Symptoms have continued despite conservative management. Pain affects walking, stairs and daily activities. Surgery is requested due to persistent functional limitation and failure of non surgical treatment.

Scenario for Class Discussion

A primary care provider orders a lumbar MRI for an adult patient with four weeks of back pain. The note says only low back pain, MRI requested. The scheduler checks that the patient has active insurance but does not check whether prior authorization is required. The MRI is scheduled. The payer later denies because PA was required and conservative therapy documentation was missing.

The trainer should ask learners what failed. The correct answer is not one single thing. Insurance eligibility was checked, but PA requirement was not checked. The provider note did not contain enough clinical detail. The scheduler moved ahead without authorization confirmation. The workflow did not stop the process before service. This scenario teaches that PA failure is usually a chain problem.

Role Play Activity

One learner acts as the PA coordinator. The trainer acts as the provider. The provider says: I ordered the MRI because I think the patient needs it. Why are you asking me for more documentation?

The PA coordinator should explain: The payer will not approve based only on the order. I need the note to show symptom duration, conservative treatment, neurological findings, red flags if present and functional impact. Without those details, the request may deny even if the MRI is clinically reasonable.

This role play teaches communication skill. PA coordinators often need to ask busy providers for more information. The tone should not sound accusatory. It should connect documentation to approval and patient access.

Prompt Engineering Practice for This Micro Module

Prompt engineering in this course should be taught as clear instruction writing, not as a technical trick. The learner should know how to ask for a safe, deidentified explanation or checklist. The prompt must never include patient identifiers. The best prompts define the role, task, context, desired output and safety rule.

Practice prompt: Act as a prior authorization trainer. Explain to a new PA coordinator why a medically appropriate service can still be denied by a payer. Use a lumbar MRI example and explain the difference between clinical need, payer criteria, documentation and approval. Do not include patient identifiers.

Learner Reflection Questions

- What is the difference between clinical need and payer approval?
- Why is doctor ordered it not enough for many prior authorization requests?
- What documentation would make a lumbar MRI request stronger?
- What should the PA coordinator do when the provider note is vague?

Micro Module Summary

Prior authorization is a payer review process before selected care is covered. It does not remove physician judgement, but it requires the provider team to prove that the request meets payer criteria. The most important idea in this micro module is that documentation connects clinical need to payer approval. Learners should leave this section knowing that PA success begins before submission, when the team checks criteria and strengthens the record.

Micro Module 1.1 Quiz

Question 1: Prior authorization is best described as:

- A. A patient right to receive any treatment they request
- B. A payer review process before selected services, medications or procedures are covered
- C. A government approval required for every medical service
- D. A billing correction process after a claim has already denied

Correct answer: B

Why A is incorrect: Patient preference matters for care planning, but it does not define payer coverage approval.

Why B is correct: Prior authorization is a payer process used before selected items or services are covered under the plan.

Why C is incorrect: Prior authorization is not required for every service and is not always direct government approval.

Why D is incorrect: Claim correction happens after billing. Prior authorization normally occurs before the service or medication is covered.

Question 2: Why can a payer deny a request even when the provider believes the care is medically necessary?

- A. Because providers are not allowed to make treatment decisions
- B. Because the payer may not see documentation proving the request meets coverage criteria
- C. Because patients cannot appeal denials
- D. Because prior authorization always means the service is not covered

Correct answer: B

Why A is incorrect: Providers still make clinical decisions. The payer decides whether the request meets coverage criteria.

Why B is correct: A payer reviewer can only assess what is submitted. Missing or vague documentation can lead to denial.

Why C is incorrect: Patients may have appeal rights depending on plan type and situation.

Why D is incorrect: Prior authorization does not automatically mean the service is excluded. It means review is required before coverage.

Question 3: Which documentation is strongest for a medication prior authorization?

- A. Patient needs this drug urgently

- B. Doctor recommends this medication
- C. Confirmed diagnosis, required prior therapy tried, response documented and supporting records attached
- D. Patient prefers this medication over the covered option

Correct answer: C

Why A is incorrect: Urgency may matter, but urgency alone does not prove criteria.

Why B is incorrect: A physician recommendation is important, but it needs supporting facts.

Why C is correct: This option provides diagnosis, treatment history, response and evidence, which are core PA elements.

Why D is incorrect: Patient preference alone rarely proves medical necessity or coverage criteria.

Micro Module 1.2: The People Involved in Prior Authorization and Why Handoffs Fail

Introduction

Prior authorization is not handled by one person. It is a shared process involving the payer, provider, PA coordinator, patient, pharmacy and often the pharmacy benefit manager. Because several people touch the workflow, failures often happen during handoffs. One person assumes another person completed the missing step. The provider assumes the PA team knows the prior therapy history. The PA coordinator assumes the provider note contains enough detail. The pharmacy assumes the authorization is linked to the correct benefit and pharmacy pathway. The patient assumes approval means the medication can be picked up immediately.

This micro module teaches learners to understand prior authorization as a team process. When learners understand the roles, they stop treating denials as random events. They begin to see where the workflow needs clearer ownership, better documentation and more reliable tracking.

Detailed Teaching Explanation

The payer is the organization that sets coverage policy and reviews the prior authorization request. The payer may be a commercial insurance company, Medicare Advantage plan, Medicaid managed care organization or another payer type. The payer reviewer checks whether the submitted information meets coverage requirements. The payer may approve, deny, pend or request more information. The payer may also apply plan rules such as step therapy, formulary restrictions, site of care requirements, network rules, medical necessity criteria and benefit limitations.

The provider is the clinician who orders the service or medication and documents the clinical rationale. This role is essential. The provider record is the source of truth for diagnosis, severity, symptoms, treatment history, contraindications and clinical need. PA coordinators cannot safely invent or assume clinical facts. If the chart does not show that a patient failed methotrexate, the PA coordinator cannot write that the patient failed methotrexate simply because the provider says it verbally in passing. The information must be supported in the record.

The PA coordinator or revenue cycle team member turns clinical information into a payer ready submission. This requires more than typing information into a form. The coordinator must check whether PA is required, identify the correct payer pathway, gather chart notes, confirm codes, review payer questions, attach supporting documents, track the request and communicate missing information to the provider.

The patient is often the person who feels the impact most directly. A delayed medication can mean uncontrolled symptoms, disease progression, anxiety, additional calls to the clinic and loss of trust. Patients may also have appeal rights and may be asked to provide consent or additional information. However, the patient should not be expected to fix documentation gaps that belong to the provider record.

The pharmacy and pharmacy benefit manager are especially important for medication PA. A medication may be approved under one benefit pathway but fail at dispensing if the wrong pharmacy is used, the wrong quantity is entered, the authorization is tied to a different pharmacy or the pharmacy benefit requirements are not satisfied.

Expanded Example

A dermatologist prescribes dupilumab for severe atopic dermatitis. The payer requires documentation that the patient tried topical corticosteroids and topical calcineurin inhibitors or that those treatments were contraindicated. The provider note says failed topical therapy but gives no medication names, dates, duration, response or adverse effects. The PA coordinator submits the request using the available note. The payer denies for missing prior therapy documentation. The patient calls the clinic because the pharmacy cannot dispense the medication.

This is a classic handoff failure. The required information existed in conversation or memory but not in payer ready documentation. The fix is not to blame one person. The fix is to redesign the workflow. Before submitting a biologic medication PA, the coordinator should check whether prior therapy history is complete. If not, the provider should document medication names, dose, duration, response, adverse effects and reason for stopping.

Tool Context in This Micro Module

This is the point where the AMA and CMS resources can be introduced lightly, but not taught fully yet. The AMA resource helps learners see the practice burden created when handoffs fail. CMS resources help learners see that PA is part of a formal coverage and operational process that requires supporting documentation and clear timing. The instructor can open the AMA page to show how prior authorization creates practice burden, then open the CMS resource to show official terminology around prior authorization and pre claim review.

The key teaching point is that public resources support understanding. They do not replace the provider note, the payer portal or the organization's internal workflow. Learners should use them to understand concepts, not to make patient specific coverage decisions.

Role Play Activity

Set up a three person role play. The trainer acts as the physician, one learner acts as the PA coordinator and another learner acts as the pharmacy staff member.

Physician: I prescribed dupilumab last week. Why is the patient still waiting?

Pharmacy: The medication will not process because prior authorization is still pending.

PA Coordinator: The request is pending because the payer needs prior topical therapy details. I need the chart updated with medication names, duration, response and reason therapy failed. Once I have that, I can complete the response and track the medication PA workflow.

Debrief the role play by asking learners what each actor needed. The physician needed to understand why documentation mattered. The pharmacy needed status. The coordinator needed verified facts. The patient needed timely access. The payer needed criteria evidence.

Scenario Questions for Learners

- If the provider says the patient failed therapy but the chart does not show details, what should the coordinator do?
- What information does the pharmacy need to help avoid a dispensing delay?
- Why is it risky for the PA coordinator to guess medication dates or dose?
- What part of the workflow should stop an incomplete request before submission?

Micro Module Summary

Prior authorization is a shared process. The payer reviews the request, the provider documents the clinical facts, the PA coordinator submits and tracks the request, the pharmacy and PBM influence medication access, and the patient experiences the result. Denials often happen when information does not move properly between these actors. AMA and CMS resources help learners understand the burden and official process language, but they do not replace internal workflows or patient specific payer rules.

Micro Module 1.2 Quiz

Question 1: Who is responsible for documenting the clinical rationale for a requested medication?

- A. The payer reviewer
- B. The treating provider
- C. The pharmacy alone
- D. The patient alone

Correct answer: B

Why A is incorrect: The payer reviews submitted information but does not create the provider record.

Why B is correct: The treating provider must document diagnosis, severity, treatment history and clinical rationale.

Why C is incorrect: The pharmacy supports dispensing and medication access but does not replace provider documentation.

Why D is incorrect: The patient may report history, but the clinical rationale must be documented in the provider record.

Question 2: A biologic PA is denied because prior therapy history is missing. What is the best next action?

- A. Resubmit the same request immediately
- B. Ask the provider to document medication names, dose, duration, response and reason stopped
- C. Tell the patient the medication can never be approved
- D. Change the diagnosis code without reviewing the chart

Correct answer: B

Why A is incorrect: Resubmitting the same incomplete information usually leads to another denial.

Why B is correct: Step therapy or prior therapy denials require specific documented treatment history.

Why C is incorrect: Many medication PA denials can be corrected when missing documentation is supplied.

Why D is incorrect: Changing codes without clinical support can create accuracy and compliance risk.

Question 3: What is a realistic limitation of AMA and CMS prior authorization resources?

- A. They cannot submit a patient specific PA request or replace payer portal review
- B. They cannot teach prior authorization concepts
- C. They always guarantee approval
- D. They remove the payer from the process

Correct answer: A

Why A is correct: AMA and CMS resources are public educational and policy references. They do not submit patient specific PA requests.

Why B is incorrect: They are useful for teaching PA burden, definitions and policy direction.

Why C is incorrect: No educational resource can guarantee approval.

Why D is incorrect: The payer still reviews and decides patient specific requests.

Micro Module 1.3: Why Prior Authorization Requests Fail

Introduction

Prior authorization failure is often predictable. It is rarely just bad luck. Requests fail because something required by the payer was missing, unclear, late, mismatched or unsupported. This micro module teaches learners how to separate the type of failure before choosing the response. If the learner cannot classify the failure, they may choose the wrong fix.

This section is important because PA teams sometimes respond emotionally to denials. They may say, the payer is denying everything, or just resubmit it. That approach wastes time. A professional denial response starts by asking what type of problem caused the denial. Was it administrative? Was it clinical? Was it a timing issue? Was it a benefit exclusion? Was it a pharmacy routing issue? Was it a documentation gap? Each problem needs a different response.

Detailed Teaching Explanation

Administrative failures are process or data problems. These include wrong CPT code, wrong ICD 10 code, missing attachment, incorrect provider information, inactive eligibility, out of network provider, wrong payer form, duplicate request or authorization not obtained before service. Administrative failures can often be corrected, but they still create delay and rework.

Clinical documentation failures happen when the submitted record does not prove medical necessity or payer criteria. These include missing failed therapy history, vague diagnosis, no objective evidence, no functional limitation, no severity score, no lab support, no imaging report, no contraindication documentation or no explanation of why a lower cost alternative is unsuitable. Clinical documentation failures usually require provider input because the missing evidence must come from the medical record.

Timing failures happen when the team acts too late. For example, a medication PA is started after the patient arrives at the pharmacy, or an imaging study is completed before authorization is obtained. Timing failures are difficult because the care may already have been delivered. If the payer does not allow retrospective authorization, the organization may face claim denial or write off.

Coverage failures happen when the service is not covered under the patient plan or the requested item falls outside policy. These cases require careful review because sometimes the denial is correct under the benefit plan, but sometimes the payer has applied the wrong policy or used outdated criteria. A strong professional does not assume either way. They verify.

Medication PA failures often combine several issues. A request may be denied because step therapy is missing, but the deeper problem may be poor treatment history documentation. A specialty medication may be approved, but the pharmacy claim may still fail because the wrong pharmacy benefit pathway was used.

Expanded Scenario

A gastroenterology clinic submits a medication PA for a biologic used in Crohn's disease. The payer denies because preferred therapy was not documented. The chart contains one sentence: Patient failed prior therapy. This is not enough. The payer needs the name of the prior medication, dose, duration, response, adverse effect if any and reason for discontinuation. The request failed because the clinical documentation did not prove the step therapy pathway.

The correct response is not to write a long emotional appeal. The correct response is to build the missing evidence. The coordinator should ask the provider to document the exact medication history. If the patient had a contraindication to the preferred therapy, the contraindication must be documented. Once the record is complete, the team can resubmit, respond to the payer request or appeal depending on the payer process.

Now compare that with an administrative failure. Suppose the same request was denied because the diagnosis code was entered incorrectly. If the chart supports the correct diagnosis, the response may be a corrected submission, not a full clinical appeal. The professional must not overcomplicate a soft denial or under respond to a hard denial.

How AMA and CMS Resources Support Failure Prevention

The AMA resource can be used to show learners why prior authorization delays and administrative burden are not imaginary. This is useful for class discussion and professional context. It helps learners explain the burden of manual PA work to providers and administrators.

The CMS resource can be used to teach formal process concepts. For example, CMS materials distinguish prior authorization and pre claim review in timing and purpose. This helps learners understand that timing matters. If the required review happens too late, the workflow risk increases. CMS resources also help learners understand that supporting medical documentation is central to coverage review.

Neither resource tells the learner whether one specific patient will be approved. Instead, both resources help the learner understand the structure of the problem and build safer workflow habits.

Prompt Engineering Practice

Prompt engineering for this micro module should be used to create checklists and classify deidentified problems. The learner should not paste real denial letters with patient identifiers. A safe practice prompt would be: Create a deidentified checklist for a medication PA denied due to missing step therapy documentation. Include medication name, dose, start date, stop date, duration, response, adverse effect, contraindication and provider verification points. Do not invent facts.

Role Play Activity

Trainer as payer reviewer: The request is denied because required preferred therapy was not documented.

Learner as PA coordinator: I will review the chart for the preferred therapy name, dose, duration, response and reason for discontinuation. If those details are missing, I will request provider documentation before resubmission.

Trainer as provider: The patient told me they tried it.

Learner response: That history needs to be documented in the record with details. The payer will not approve based on an undocumented statement.

Debrief: The learner should be praised for not guessing, not blaming and not resubmitting too quickly. The skill being practiced is documentation discipline.

Learner Practice Questions

- Is the denial administrative, clinical, timing based or coverage based?
- What information is missing from the record?
- Who must supply the missing information?
- Can this be corrected by resubmission, or does it require appeal?
- What should be tracked after the response is submitted?

Micro Module Summary

Prior authorization requests fail for predictable reasons. Administrative failures involve data, forms, eligibility, coding or process errors. Clinical documentation failures involve missing evidence of medical necessity, failed therapy, severity or objective support. Timing failures happen when the PA check is missed or when requests are not tracked. Coverage failures require policy review. AMA and CMS resources help learners understand the broader burden and official process concepts, but humans must still verify the chart and follow payer specific rules.

Micro Module 1.3 Quiz

Question 1: Which example is mainly an administrative PA failure?

- A. Missing CPT code on the request
- B. No documented failed therapy history
- C. No functional limitation described

D. No objective evidence of disease severity

Correct answer: A

Why A is correct: A missing CPT code is a data or coding issue and is mainly administrative.

Why B is incorrect: Failed therapy history is clinical documentation.

Why C is incorrect: Functional limitation is clinical documentation that supports medical necessity.

Why D is incorrect: Objective evidence is clinical support, not simply administrative data.

Question 2: Which response is strongest for a step therapy denial?

A. The provider prefers the requested drug

B. The patient is upset about the denial

C. The record shows the required medication name, dose, duration, response and reason stopped

D. The coordinator resubmits the same form quickly

Correct answer: C

Why A is incorrect: Provider preference alone usually does not satisfy step therapy criteria.

Why B is incorrect: Patient frustration is understandable but does not prove medical necessity.

Why C is correct: Step therapy denials require exact treatment history or documented exception.

Why D is incorrect: Fast resubmission without better evidence usually repeats the denial.

Question 3: What should a coordinator do when a payer question asks whether prior therapy failed?

A. Answer yes if the provider probably remembers it

B. Answer yes only if the chart supports the prior therapy history

C. Leave the question blank and submit

D. Use the same answer for every patient with the same diagnosis

Correct answer: B

Why A is incorrect: Guessing creates accuracy and compliance risk.

Why B is correct: Clinical answers must be supported by the provider record.

Why C is incorrect: Blank required fields can lead to denial or pending status.

Why D is incorrect: Each PA request must reflect the individual patient record.

Micro Module 1.4: Using AMA and CMS Resources in a Prior Authorization Learning Workflow

Introduction

This micro module is the full tool lesson for Module 1. By this point, the learner understands what prior authorization means, who is involved and why requests fail. Now the learner can understand how public PA resources fit into the learning process.

The earlier tool, CoverMyMeds, was medication workflow specific, but it caused access concerns because it may not reopen in India and may require registration. For this revised module, the tool section now uses AMA Prior Authorization resources and CMS Prior Authorization resources. These are public reference tools. They are not patient specific submission systems. They are used because they are free, globally accessible for reading, and directly relevant to prior authorization education.

What the Tools Are

AMA Prior Authorization Resources

The AMA prior authorization resource page is an educational and advocacy resource focused on prior authorization burden, reform initiatives, practice resources, research and reports. It is useful for explaining why prior authorization causes delays, administrative workload and frustration in healthcare practices.

CMS Prior Authorization Resources

CMS prior authorization resources are official public resources explaining prior authorization, pre claim review and electronic prior authorization concepts. They help learners understand formal terminology, timing, documentation requirements and the direction of digital PA processes.

Why These Tools Are Better for This Module

Module 1 is a foundation module. The learner does not yet need a private submission portal. The learner needs language, concepts, definitions and workflow understanding. AMA and CMS resources fit that purpose better because they can be opened without account registration, do not require learner access to a provider account and do not require real patient data.

These resources are also safer for international learners. A learner in India or outside the United States can open the pages, read the material and participate in the learning activity without needing provider credentials or payer portal access.

Competitors and Alternatives

Alternatives include payer portals, PBM portals, electronic prior authorization tools, EHR embedded ePA tools, phone based PA, fax based PA and commercial medication PA platforms. These may be relevant in real operations, but they are not ideal for a foundational international training module because access may require registration, payer contracts, provider credentials, country specific login or patient data.

AMA and CMS resources are selected here because they are public, free, accessible and educational. They help learners understand prior authorization before they touch patient specific workflows.

Clear Limitation

AMA and CMS resources do not submit a prior authorization request. They do not track a payer decision. They do not verify a patient benefit. They do not replace a payer portal, EHR, pharmacy system or organizational workflow. They are used to teach concepts, improve language, support workflow design and help learners understand why documentation and timing matter.

Step Based Workflow: How to Use AMA and CMS Resources in Training

The instructor can demonstrate the workflow in Camtasia using a browser screen recording. No login is needed and no patient data is used.

1. Open a browser.
2. Search for AMA prior authorization.
3. Open the AMA prior authorization page.
4. Show the learner the page sections such as overview, reform initiatives, practice resources, research and reports.
5. Pause on the description of prior authorization as a cost control process and discuss how it affects payment and patient access.
6. Ask learners to identify three workflow problems mentioned or implied by the AMA resource, such as delay, burden and lack of transparency.
7. Open a new browser tab.
8. Search for CMS prior authorization resources or CMS prior authorization and pre claim review.
9. Open the CMS resource page.
10. Show how CMS explains the timing of prior authorization and pre claim review.

11. Pause on the idea that supporting medical documentation is submitted for review.
12. Ask learners to connect CMS terminology to a real workflow: request, documentation, review, decision and next action.
13. Create a simple learner worksheet with three columns: resource finding, what it means in practice, workflow action.
14. Complete one example together: CMS says supporting documentation is required. In practice, the coordinator must not submit vague notes. Workflow action: verify diagnosis, failed therapy, objective evidence and attachments before submission.

Camtasia Demonstration Script

Video title: Using AMA and CMS Resources to Understand Prior Authorization

Estimated length: 5 to 7 minutes

Voiceover script: In this demonstration, we are not submitting a real prior authorization request. We are using public resources to understand the prior authorization problem. First, we open the AMA prior authorization resource page. This helps us understand the practice burden, patient delay and reform conversation. Next, we open the CMS prior authorization resources. This helps us understand formal process language, timing and supporting documentation. These resources are useful because they are free, public and do not require learner login. They help us build safe workflow understanding before we ever touch a patient specific payer portal.

Tool Based Case Study 1: The Lumbar MRI That Was Scheduled Too Early

Characters

Aisha: PA Coordinator

Dr. Mehra: Primary Care Physician

Leo: Scheduling Lead

Nina: Patient Access Manager

Scenario

A primary care clinic schedules a lumbar MRI for a patient with four weeks of back pain. The scheduler verifies active insurance but does not check whether prior authorization is required. The provider note says low back pain, MRI requested. The payer later denies because prior authorization was required and conservative therapy documentation was missing.

How the Tools Are Used

Nina asks Aisha to prepare a short teaching session for the scheduling team. Aisha opens the AMA prior authorization resource page and uses it to explain that prior authorization is a payer cost control process that can delay care when workflows are incomplete. She then opens the CMS resource and shows the team how prior authorization happens before services are rendered and depends on supporting documentation.

Aisha creates a three point workflow reminder for Leo's scheduling team: check whether PA is required before scheduling, check whether the provider note contains the required clinical details, and do not mark the imaging appointment ready until authorization status is known.

Teaching Point

The AMA resource helps the team understand the burden and patient access risk. The CMS resource helps the team understand timing and documentation. Together, they help turn a denial into a workflow lesson.

Learner Task

Learners should write the revised scheduling checklist: payer checked, PA requirement checked, clinical documentation checked, owner assigned, authorization status recorded.

Tool Based Case Study 2: The Biologic Medication Denial

Characters

Sara: Rheumatology PA Coordinator

Dr. Raman: Rheumatologist

Imran: Pharmacy Liaison

Veda: Practice Manager

Scenario

A rheumatology clinic submits a medication PA for adalimumab. The payer denies because methotrexate failure is not documented. The provider says the patient tried methotrexate, but the chart only says failed prior therapy. The patient is waiting at the pharmacy and calls twice in one day.

How the Tools Are Used

Veda asks Sara to explain the issue to the team. Sara uses the AMA resource to show how PA burden affects patients and staff. She then uses the CMS resource to reinforce that supporting medical documentation is central to review. Sara explains that the denial was not only a payer problem. The failure was the missing documentation bridge between clinical need and payer criteria.

Sara then creates a documentation request for Dr. Raman: medication name, dose, start date, stop date, duration, response, adverse effects and reason therapy stopped. Imran is asked to update the pharmacy once a decision is received.

Teaching Point

The public resources help the team understand why vague documentation creates delay. They do not provide the patient specific answer. The chart must provide that answer.

Learner Task

Learners should identify the missing documentation and draft a professional message to the provider requesting the exact therapy history needed.

Tool Based Case Study 3: Approval Does Not Equal Access

Characters

Maya: Oncology Coordinator

Dr. Shah: Oncologist

Rita: Specialty Pharmacy Representative

Owen: Revenue Cycle Lead

Scenario

An oncology medication is approved, but the pharmacy claim rejects because the approval is linked to a different specialty pharmacy. The patient is anxious because treatment was expected to begin this week.

How the Tools Are Used

Owen uses the AMA resource to frame the access issue for the team: prior authorization burden is not only paperwork, it can delay treatment. Maya uses CMS terminology to map the process: request, supporting information, review, decision and post decision action. The team realizes that decision tracking alone was not enough. Pharmacy routing also needed to be part of the workflow checklist.

Teaching Point

The tools help the team redesign the workflow after the failure. The checklist now includes benefit pathway, approved pharmacy, authorization dates, quantity limit and dispensing route.

Learner Task

Learners should update the medication access checklist so that approval details are verified before the patient is told the medication is ready.

Tool Based Role Play

Trainer as physician: I ordered the medication. Why are we discussing AMA and CMS pages instead of just submitting the request?

Learner as PA coordinator: These resources do not submit the request. We are using them to understand the process and avoid preventable mistakes. The AMA resource helps explain the burden and patient delay. The CMS resource helps us understand timing and supporting documentation. For this patient, I still need the provider record to show medication name, dose, duration, response and reason stopped before the payer can review the request properly.

Trainer as physician: Can you just say the patient failed therapy?

Learner response: I cannot answer yes unless the chart supports it. If we submit unsupported information, the request may deny again and the record may be inaccurate. I need verified documentation before responding.

Tool Based Learner Questions

- What does the AMA resource help us understand about PA burden?
- What does the CMS resource help us understand about process timing and documentation?
- Why are these resources safer for international training than a restricted submission tool?
- What can these resources not do?
- How can the learner convert a public resource finding into a workflow action?

Micro Module Summary

AMA and CMS resources replace CoverMyMeds in Module 1 because they are public, free and easier for international learners to open without registration barriers. AMA helps learners understand prior authorization burden, delays, physician practice impact and reform language. CMS helps learners understand formal process language, timing, electronic PA direction and documentation concepts. These resources do not submit PA requests or replace payer portals. Their value in Module 1 is educational: they help learners build safer, clearer and more disciplined PA workflows before using patient specific tools.

Micro Module 1.4 Quiz

Question 1: AMA Prior Authorization resources are best used in this module for:

- A. Submitting patient specific PA requests
- B. Understanding PA burden, delays, practice impact and reform language
- C. Replacing payer portals
- D. Guaranteeing payer approval

Correct answer: B

Why A is incorrect: AMA resources are public educational and advocacy resources, not submission portals.

Why B is correct: They help learners understand the real world burden and reform context of prior authorization.

Why C is incorrect: They do not replace payer portals or patient specific workflows.

Why D is incorrect: No public resource can guarantee approval.

Question 2: CMS Prior Authorization resources are best used for:

- A. Understanding official process language, timing and supporting documentation concepts
- B. Writing prescriptions
- C. Dispensing medication
- D. Automatically approving claims

Correct answer: A

Why A is correct: CMS resources help learners understand formal terminology and process concepts.

Why B is incorrect: Prescribing is a provider function.

Why C is incorrect: Dispensing is handled by pharmacies.

Why D is incorrect: CMS resources do not automatically approve claims.

Question 3: Why are AMA and CMS resources suitable for international learners in Module 1?

- A. They require provider credentials from every country
- B. They are public resources that can be used without normal learner registration
- C. They replace all payer specific rules
- D. They store patient medical records

Correct answer: B

Why A is incorrect: These public pages do not require normal learner provider credentials for reading.

Why B is correct: They are accessible public references and do not require country restricted PA tool registration.

Why C is incorrect: They do not replace payer specific rules or local policy.

Why D is incorrect: They should not store patient records and are used here for education only.

Module 1 Full Summary

Module 1 establishes the foundation for all later modules. The learner now understands that prior authorization is not a simple form or a random payer obstacle. It is a structured coverage review process where clinical need must be translated into payer ready documentation. A request may be clinically reasonable but still fail if it does not prove the payer criteria.

The module also teaches that PA is a team workflow. The provider documents the clinical facts. The coordinator checks requirements, submits and tracks the request. The payer reviews against policy. The pharmacy and PBM affect medication access. The patient experiences the delay, approval or denial. When one handoff fails, the entire process can break.

Learners also learned to classify failures. Administrative failures involve codes, forms, eligibility, dates or missing attachments. Clinical documentation failures involve weak medical necessity, missing failed therapy history, vague notes or lack of objective support. Timing failures happen when the PA check is missed or when pending requests are not tracked. The correct response depends on the type of failure.

Finally, learners practiced using AMA and CMS public resources as Module 1 reference tools. AMA is used because it explains prior authorization burden, practice impact and reform language. CMS is used because it explains formal process concepts, timing and supporting documentation. These tools are free public resources and more suitable for international access than a restricted submission platform. Their limitation is clear: they do not replace payer portals, provider documentation, benefit verification or patient specific workflows.

Hands On Learning After Module 1

Hands On Case Study 1: Missing Step Therapy Documentation

Scenario: A rheumatology clinic submits a medication PA for adalimumab. The payer denies the request because methotrexate failure is not documented. The provider says the patient tried methotrexate in the past, but the chart only says failed prior therapy. The patient is waiting at the pharmacy and calls the clinic twice in one day.

Learner Questions

- What is the real reason for the denial?
- Is the problem administrative, clinical documentation based, timing based or coverage based?
- What exact information must the provider document before resubmission?
- How can AMA and CMS resources help explain this issue to the team?
- What should be recorded after the payer decision is received?

Model Answer

The denial is mainly a clinical documentation failure because the payer did not receive proof of step therapy. The phrase failed prior therapy is not enough. The provider should document the medication name, dose, start date, stop date, duration, response, adverse effects if any and reason for discontinuation. AMA resources can help explain the burden and delay caused by incomplete PA workflows. CMS resources can help reinforce that supporting documentation is central to review. After the payer decision, the coordinator should record the status, authorization number if approved, approval dates, drug details, pharmacy details, renewal date or denial reason if not approved.

Hands On Case Study 2: Fax Failure and Delayed Specialty Medication

Scenario: A dermatology practice sends a dupilumab PA by fax. Seven days later, the patient calls because the pharmacy still cannot fill the medication. The coordinator calls the payer and is told the fax was never received. The provider is upset because the medication was supposed to start last week.

Learner Questions

- What failed in the workflow?
- Why is fax risky for medication prior authorization?
- How can the AMA resource help explain the practice burden?
- How can the CMS resource help explain process timing and documentation?
- What status updates should the coordinator monitor after submission through the organization's approved workflow?

Model Answer

The workflow failed because the practice had no reliable submission visibility. Fax can fail silently, and even when a fax confirmation exists, it may not prove the payer processed the request correctly. The AMA resource helps explain how manual PA processes create administrative burden and patient delay. The CMS resource helps explain why timing and supporting documentation matter. The coordinator should monitor whether the request is pending, approved, denied or waiting for more information through the organization's approved payer or EHR workflow.

Hands On Case Study 3: Approval Does Not Lead to Dispensing

Scenario: An oncology medication PA is approved, but the pharmacy claim rejects because the authorization is linked to a different specialty pharmacy. The patient is anxious because treatment was expected to begin this week.

Learner Questions

- Why can a medication be approved but still fail at the pharmacy?
- What pharmacy or benefit details should be checked before submission?
- What should the coordinator do immediately after discovering the problem?
- How should this case change the future PA checklist?

Model Answer

A medication can be approved but still fail if pharmacy routing, benefit pathway, quantity, authorization dates or dispensing pharmacy details do not match. The coordinator should check specialty pharmacy requirements, pharmacy identifiers where applicable, payer pathway, quantity and approval terms. After discovering the problem, the coordinator should contact the payer urgently through the approved workflow, document the representative name, reference number and correction instructions, then coordinate with the pharmacy. The future checklist should include specialty pharmacy confirmation before submission and after approval.

Suggested Group Role Play

Assign three learners: provider, PA coordinator and payer reviewer. The payer reviewer says the medication is denied because required therapy history is missing. The provider says the patient already tried the required drug. The PA coordinator must respond professionally and request chart level detail without sounding confrontational.

Expected coordinator response: I understand the patient has tried the required drug, but the payer needs it documented in the record. Please confirm medication name, dose, start date, stop date, duration, response and reason stopped. Once the chart is updated, I will respond through the approved medication PA workflow and track the decision.

Debrief point: The coordinator protects the patient and provider by refusing to guess. The role play should show confidence, clarity and respect.

End of Module 1 Questions

Question 1: A payer denies a medication PA because step therapy was not documented. What is the first evidence the team should gather?

- A. Patient payment receipt
- B. Complete medication history with drug name, dose, dates, duration, response and reason stopped
- C. Provider business card
- D. A new form with no clinical changes

Correct answer: B

Why A is incorrect: Payment information does not prove step therapy.

Why B is correct: Step therapy denials require exact treatment history or a documented exception.

Why C is incorrect: Provider identity is not the missing clinical evidence.

Why D is incorrect: A new form without stronger documentation will likely repeat the denial.

Question 2: What is the strongest reason for using AMA and CMS resources in Module 1?

- A. They are generic chatbots
- B. They are public, free educational resources that help learners understand PA burden, process language and documentation risk
- C. They replace payer medical review
- D. They write all denial appeals automatically

Correct answer: B

Why A is incorrect: AMA and CMS resources are not generic chatbots.

Why B is correct: They fit the foundational learning goal and can be opened without restricted registration.

Why C is incorrect: The payer still reviews and decides.

Why D is incorrect: They are not automatic appeal writing tools.

Question 3: Which action creates the highest privacy risk during training?

- A. Using a fake patient scenario
- B. Discussing a deidentified medication denial
- C. Uploading a real denial letter with patient name, member ID and diagnosis into an unapproved tool
- D. Practicing with fictional payer questions

Correct answer: C

Why A is incorrect: Fake scenarios are appropriate for training.

Why B is incorrect: Deidentified examples reduce privacy risk.

Why C is correct: Real identifiers in an unapproved tool create serious privacy and compliance risk.

Why D is incorrect: Fictional payer questions are safe for practice.

Question 4: A request is denied because the CPT code is missing. What type of failure is this mainly?

- A. Administrative failure
- B. Experimental treatment failure
- C. Step therapy failure
- D. Peer to peer failure

Correct answer: A

Why A is correct: A missing CPT code is an administrative or coding issue.

Why B is incorrect: Experimental treatment denials involve clinical or policy arguments.

Why C is incorrect: Step therapy involves prior treatment history.

Why D is incorrect: Peer to peer is a review process, not the root failure here.

Question 5: What does a PA coordinator prove by tracking the status after submission?

- A. That the provider no longer needs to document clinical facts
- B. That the request has moved to a known outcome such as pending, approved, denied or needing more information
- C. That approval is guaranteed
- D. That the pharmacy will always dispense immediately

Correct answer: B

Why A is incorrect: Provider documentation remains necessary.

Why B is correct: Status tracking prevents requests from disappearing or missing additional information deadlines.

Why C is incorrect: Tracking does not guarantee approval.

Why D is incorrect: Dispensing still depends on pharmacy, benefit and authorization details.

Instructional Designer Notes After Module 1

This section is for the instructional designer and production team. It explains how to convert Module 1 into a digital learning experience using Rise, Camtasia, Storyline, animation and chatbot based practice. These notes should not be shown as learner facing content unless the course owner wants a facilitator guide version.

Overall Design Strategy

Module 1 should not be built as a simple page of text. The learning experience should move from concept to workflow to decision making to public resource practice. Rise can be used for clean content delivery, knowledge checks and structured reading. Camtasia can be used for short narrated walkthroughs of AMA and CMS public resources. Storyline should be used for decision based scenarios, branching practice and interactive role plays. Animation should be used to show workflows, handoffs and failure points. A chatbot can be used as a safe practice assistant where learners rehearse how to ask for missing documentation and how to classify denial causes.

Where Rise Should Be Used

- Use Rise for the main module shell with four lessons, one for each micro module.
- Use a Process block for practical PA thinking points: check requirement, verify eligibility, gather documentation, complete payer questions, submit, track status and record outcome.
- Use Accordion blocks for the five PA actors: provider, PA coordinator, payer, pharmacy and patient.
- Use Flashcard blocks for terms such as prior authorization, medical necessity, payer criteria, step therapy, soft denial, hard denial and pharmacy benefit manager.
- Use Knowledge Check blocks after each micro module for the three MCQs with full feedback.
- Use a labeled graphic block for the medication PA workflow showing provider, coordinator, payer, pharmacy, PBM and patient.

Where Camtasia Should Be Used

- Camtasia video 1: What prior authorization means. Use simple animated slides showing provider order moving to payer review and then approval or denial.
- Camtasia video 2: Why handoffs fail. Show how the provider, coordinator, payer, pharmacy and patient pass information and where the chain breaks.
- Camtasia video 3: Using AMA Prior Authorization resources. Show the page, key sections, and how the learner converts the resource into workflow discussion points.
- Camtasia video 4: Using CMS Prior Authorization resources. Show the resource page, timing concepts, and the role of supporting documentation.
- Camtasia video 5: How not to answer payer questions. Show a coordinator about to guess prior therapy history, then pause and correct the behavior by checking the chart and asking the provider.

Stepwise Camtasia Guide for AMA Resource

15. Open the browser.
16. Search for AMA prior authorization.
17. Open the AMA Prior Authorization page.
18. Point out the overview and practice resource sections.
19. Highlight the idea that prior authorization creates delays, practice burden and patient access concerns.
20. Pause and ask the learner: which part of our internal workflow can reduce this burden?
21. Show a blank worksheet: Resource finding, what it means, workflow action.
22. Fill one example: PA creates delay. Meaning: incomplete workflow causes patient access risk. Action: check PA requirement before scheduling or dispensing.

Stepwise Camtasia Guide for CMS Resource

23. Open the browser.
24. Search for CMS prior authorization resources.
25. Open the relevant CMS prior authorization or pre claim review resource page.
26. Point out the definition and timing language.
27. Explain that prior authorization normally happens before the service is rendered or covered.
28. Highlight the role of supporting medical documentation.
29. Pause and ask the learner: what documentation would support this request?

30. Fill the worksheet: CMS concept, practical meaning, workflow action.

Where Storyline Should Be Used

- Storyline activity 1: Submit or stop? The learner reviews a medication PA request. The chart says failed prior therapy but gives no details. Correct path: ask the provider for documentation.
- Storyline activity 2: What type of failure is this? The learner receives denial reasons and classifies them as administrative, clinical documentation, timing, coverage or pharmacy routing failure.
- Storyline activity 3: Role play with the physician. The physician pushes back and says just submit it. The learner chooses the best professional response.
- Storyline activity 4: Public resource to workflow action. The learner sees an AMA or CMS finding and must select the correct workflow improvement.

Where Animation Should Be Used

- Animate the difference between clinical need and payer approval. Show documentation acting as the bridge.
- Animate the five PA actors and where the handoff breaks when step therapy documentation is missing.
- Animate the fax failure scenario and compare it with a tracked electronic workflow managed inside an approved organizational system.
- Animate soft versus hard denials. Soft denial can move through correction. Hard denial requires clinical evidence and review.
- Animate the approval does not equal dispensing concept. Show approval granted, pharmacy rejection due to routing, correction and successful dispensing.

Where Chatbot Practice Should Be Used

A chatbot should be used as a safe practice coach, not as a live patient data tool. It should be trained or fed only with module content, fake scenarios, approved scripts, deidentified examples and the organization's training rules. The chatbot should not accept real PHI. Its purpose is to let learners practice asking for missing documentation, classifying PA failures and choosing the next action.

What Kind of Chatbot to Build

Build a closed knowledge practice chatbot for Module 1. The chatbot should not browse the web and should not answer outside the training scope. It should act as a PA workflow coach for fictional cases. It should respond using the module terminology: prior authorization, medical necessity, payer criteria, step therapy, administrative failure, clinical documentation failure, timing failure, public PA resources, status tracking and provider verification.

How the Chatbot Should Be Fed

Feed the chatbot with Module 1 content, glossary, case studies, MCQs, role play scripts, AMA and CMS tool explanations, privacy rules and model answers. Add a strict guardrail: Do not request, accept or process real patient identifiers. If the learner enters patient identifiers, stop and instruct them to remove PHI and use a fictional scenario.

Sample Chatbot Prompts for Learners

- I have a fake medication PA denial for missing step therapy. What should I ask the provider to document?
- Classify this fake denial: missing CPT code and no physical therapy notes. Is it soft, hard or mixed?
- Practice a role play with me. You are the physician who says, just submit it. I am the PA coordinator.
- Give me feedback on whether my response to the provider is professional and accurate.
- Turn this CMS concept into a workflow action. Do not include real patient information.

Suggested Build Map

Course Part	Best Tool	Instructional Treatment	Reason
Module introduction	Rise	Text, image block and short knowledge check	Best for structured overview

Clinical need versus payer approval	Animation inside Rise or Camtasia	Bridge animation showing documentation connecting provider and payer	Clarifies abstract concept
Five actors	Rise accordion plus animation	Expandable actor cards and handoff animation	Helps learners see responsibility
Failure classification	Storyline	Drag and drop or branching scenario	Builds decision skill
AMA resource overview	Camtasia	Narrated public page walkthrough	Shows burden and practice context
CMS resource overview	Camtasia	Narrated public page walkthrough	Shows official process and documentation language
Provider pushback role play	Storyline or chatbot	Branching dialogue or chatbot practice	Builds communication skill
Hands on cases	Storyline	Scenario with decision points and feedback	Tests applied judgement
End quiz	Rise or LMS quiz	Scored MCQs with full feedback	Checks knowledge retention

Source Notes for Factual Claims

AMA Prior Authorization resources: The AMA prior authorization page describes prior authorization as a health plan cost control process and provides reform initiatives, practice resources, research and reports. Source: <https://www.ama-assn.org/practice-management/prior-authorization>

CMS Prior Authorization and Pre Claim Review resources: CMS explains that prior authorization and pre claim review are similar but differ in timing, and that supporting medical documentation is submitted for review. Source: <https://www.cms.gov/data-research/monitoring-programs/medicare-fee-service-compliance-programs/prior-authorization-and-pre-claim-review-initiatives>

CMS electronic prior authorization resources: CMS provides public information on electronic prior authorization and related health system concepts. Source: <https://www.cms.gov/priorities/electronic-prior-authorization/overview>

HIPAA and cloud services: HHS states that covered entities and business associates may use cloud service providers to store or process ePHI only when HIPAA requirements are met, including business associate agreement requirements when applicable. Source: <https://www.hhs.gov/hipaa/for-professionals/faq/2075/may-a-hipaa-covered-entity-or-business-associate-use-cloud-service-to-store-or-process-ephi/index.html>

MICRO MODULE 1.4: Using the Public PA Resource Workflow in Practice

This micro module teaches learners how to use the Module 1 public PA resource in a practical prior authorization learning workflow. The tool is used to understand prior authorization burden, process timing, documentation expectations, workflow risk, access delays, and why incomplete information often leads to denial.

This tool is not a prior authorization submission platform. It does not check eligibility, verify benefits, submit PA requests, track payer decisions, replace payer portals, or guarantee approval. It is used as a public learning resource so learners can understand the PA process before moving into clinical documentation, denials, appeals, and advanced workflow tools in later modules.

TOOL DEMONSTRATION: Public PA Resource Workflow

Is it free?

Public access is available without normal learner registration.

Website

Use the public PA resource link selected for Module 1.

What the tool is about

The Public PA Resource Workflow is a public prior authorization reference resource used to explain PA burden, process timing, documentation expectations, administrative workload, access delays, and workflow risks.

Purpose in this module

The purpose of this tool is to help learners understand why prior authorization delays happen and how stronger documentation and workflow control can reduce preventable denials.

Why this tool is used

This tool is used because Module 1 is foundational. Learners first need to understand the PA process, terminology, burden, handoffs, documentation risk, and timing before learning patient specific submission tools in later modules.

Where it appears in the module

This tool appears in Micro Module 1.4, after learners have already studied PA meaning, role responsibilities, workflow handoffs, and common failure types.

Case studies linked to this tool

1. Lumbar MRI scheduled without PA check
2. Biologic medication denied for missing step therapy
3. Approval does not equal access

Step by Step Use Inside the Case Study

1. Open the public PA resource selected for Module 1.
2. Review the section that explains prior authorization burden, timing, supporting documentation, or process risk.
3. Identify one point related to delay, documentation, administrative workload, payer review, or workflow failure.
4. Connect that point to the case study.
5. Ask what went wrong in the workflow.
6. Ask what documentation was missing.
7. Ask who should have owned the missing step.
8. Ask what should have been tracked.
9. Convert the public resource finding into one workflow action.
10. Do not enter real patient information.
11. Do not use the public resource to make a patient specific coverage decision.
12. Do not treat the public resource as a replacement for payer rules, provider documentation, benefit verification, or internal workflows.

CASE STUDY EASY: Lumbar MRI Scheduled Without PA Check

Scenario

A primary care clinic schedules a lumbar MRI for an adult patient with four weeks of back pain. The scheduler checks active insurance but does not check whether prior authorization is required.

The provider note says only: low back pain, MRI requested.

The payer later denies because prior authorization was required and conservative therapy documentation was missing.

Task

Identify the workflow failure and create a corrected scheduling checklist.

Tool sync

The public PA resource is useful because this case is about process timing and documentation. The learner uses the resource to understand that prior authorization must be checked before the service and that supporting documentation must be ready before submission.

Step by step tool use

1. Open the public PA resource.
2. Identify one point about PA timing, administrative burden, or supporting documentation.
3. Return to the lumbar MRI case.
4. Identify where the workflow failed.
5. Identify what should have been checked before scheduling.
6. Identify what documentation was missing from the provider note.
7. Decide who should have owned the PA check.
8. Convert the resource finding into a scheduling checklist.

Model answer

The failure was mixed. Eligibility was checked, but the prior authorization requirement was not checked. The provider note was too vague because it did not include symptom duration, conservative therapy, functional impact, red flags, neurological symptoms, or imaging rationale.

The scheduler moved the appointment forward before authorization status was confirmed. The corrected workflow should stop the appointment from being marked ready until PA requirement, documentation readiness, and authorization status are confirmed.

Corrected scheduling checklist

1. Check active insurance.
2. Check whether prior authorization is required.
3. Confirm the correct payer pathway.
4. Confirm the diagnosis code.
5. Review the provider note for symptom duration.
6. Check whether conservative therapy is documented.
7. Check whether functional impact is documented.
8. Check whether red flags or neurological symptoms are documented.
9. Confirm whether imaging rationale is clear.
10. Gather required reports and attachments.
11. Assign the authorization owner.
12. Record the authorization status.
13. Track pending payer requests.
14. Mark the appointment ready only after PA status is confirmed.

Teaching point

This denial was not caused by one person. It was a workflow failure. The scheduler checked eligibility but missed the PA requirement. The provider note did not contain enough clinical detail. The appointment moved forward too early. The public PA resource helps learners understand why process timing and documentation control matter.

CASE STUDY INTERMEDIATE: Biologic Medication Denied for Missing Step Therapy

Scenario

A rheumatology clinic submits a PA for adalimumab. The payer denies because methotrexate failure is not documented.

The provider says the patient tried methotrexate, but the chart only says: failed prior therapy.

The patient is waiting at the pharmacy and calls twice in one day.

Task

Identify the denial reason and draft a provider documentation request.

Tool sync

The public PA resource is useful because this case shows how incomplete documentation creates delay, staff rework, patient frustration, and preventable denial risk. The tool helps learners understand the workflow problem. It does not supply the missing patient specific facts. The chart must supply those facts.

Step by step tool use

1. Open the public PA resource.
2. Identify one point about documentation burden or supporting information.
3. Return to the biologic medication case.
4. Identify the payer's missing requirement.
5. Separate what is known from what is missing.
6. Decide who must supply the missing information.
7. Draft a professional message to the provider.
8. Do not invent medication dates, dose, duration, response, or adverse effects.
9. Decide what should be tracked after the response is submitted.

Model answer

The denial happened because step therapy failure was not documented with enough detail. The phrase failed prior therapy is not sufficient for payer review.

The provider must document methotrexate dose, start date, stop date, duration, response, adverse effects if any, and reason therapy was discontinued. The PA coordinator should not guess or add unsupported details.

Provider message

Dr. Raman, the adalimumab PA was denied because methotrexate failure is not documented in the chart. Please update the record with methotrexate dose, start date, stop date, duration, response, adverse effects if any, and reason therapy was discontinued. Once the chart is updated, I can respond through the approved PA workflow and track the decision.

Teaching point

The problem is not only the denial. The real issue is the missing documentation bridge between clinical need and payer criteria. The public PA resource helps learners understand why incomplete documentation causes delays, but the actual correction must come from the verified provider record.

CASE STUDY HARD: Approval Does Not Equal Access

Scenario

An oncology medication PA is approved, but the pharmacy claim rejects because the authorization is linked to a different specialty pharmacy. The patient is anxious because treatment was expected to begin this week.

Task

Update the medication access checklist so approval details are verified before the patient is told the medication is ready.

Tool sync

The public PA resource is useful because this case shows that prior authorization is not only about getting approval. The workflow must also confirm the post decision access pathway.

Approval must be checked against pharmacy routing, authorization dates, quantity limits, benefit pathway, dispensing instructions, and renewal requirements.

Step by step tool use

1. Open the public PA resource.
2. Identify one point about PA complexity, process control, or access delay.
3. Return to the oncology medication case.
4. Map the workflow from request to decision to dispensing.
5. Identify what was missed after approval.
6. Identify what the pharmacy needed before dispensing.
7. Build a checklist that prevents this issue from repeating.
8. Confirm that the patient should not be told the medication is ready until access is verified.

Model answer

The medication was approved, but the dispensing pathway was not confirmed. The authorization was linked to a different specialty pharmacy, so the pharmacy claim rejected even though PA approval existed.

The coordinator should verify authorization number, approval start date, approval end date, drug name, quantity limit, benefit pathway, approved specialty pharmacy, dispensing pharmacy match, pharmacy claim status, and renewal date before telling the patient the medication is ready.

Updated medication access checklist

1. Record authorization number.
2. Check approval start date.
3. Check approval end date.
4. Verify approved drug name.
5. Verify approved dose and quantity.
6. Confirm benefit pathway.
7. Confirm required specialty pharmacy.
8. Match approved pharmacy with dispensing pharmacy.
9. Check whether the pharmacy claim processes correctly.
10. Record payer reference number if correction is needed.
11. Notify the provider of any pharmacy routing issue.
12. Notify the patient only after the access pathway is confirmed.
13. Add renewal date to the tracking system.
14. Record next action owner.

Teaching point

Approval is not always the final step. A PA can be approved but still fail at pharmacy if routing, quantity, benefit pathway, or dispensing details are wrong. The public PA resource helps learners understand access delay and process risk.

Role Play: Explaining Why the Public PA Resource Matters

PA Coordinator Aisha

Dr. Raman, I reviewed the denial and the issue is missing step therapy documentation.

Dr. Raman

The patient already tried methotrexate. Can you just write that?

PA Coordinator Aisha

I cannot write it unless the chart supports it. The payer needs the medication name, dose, dates, duration, response, and reason therapy stopped.

Dr. Raman

Why are we using this public PA resource then?

PA Coordinator Aisha

We are using it to understand the PA process, documentation burden, and workflow risk. It helps us see why incomplete records create delays. It does not replace the payer portal or the chart. The missing facts still need to come from your documentation.

Dr. Raman

What should I add?

PA Coordinator Aisha

Please document methotrexate dose, start date, stop date, duration, response, adverse effects if any, and reason for discontinuation. Once updated, I can respond through the approved workflow and track the decision.

Debrief

The coordinator should not blame the provider. The coordinator should explain the documentation gap clearly, connect it to payer review, and request exact chart level facts. The learner should show confidence, accuracy, and respect.

Learner Practice Questions

1. What part of the workflow failed in the lumbar MRI case?
2. What documentation was missing before the MRI was scheduled?
3. Why is failed prior therapy too vague for a biologic medication PA?
4. What exact details should the provider document for step therapy?
5. Why can a medication be approved but still fail at the pharmacy?
6. What should be checked before telling the patient the medication is ready?
7. What can the public PA resource teach?
8. What can the public PA resource not do?
9. Why should real patient information not be used in this activity?
10. How can a learner convert a public resource finding into a workflow action?

Quiz

Q1. What is the correct use of the public PA resource in Module 1?

- A. To submit patient specific PA requests
- B. To understand PA burden, timing, documentation expectations, and workflow risks. CORRECT
- C. To guarantee payer approval
- D. To replace provider documentation

Explanation: The resource supports learning and workflow understanding. It does not submit requests, replace payer rules, or guarantee approval.

Q2. A lumbar MRI is denied because PA was required and conservative therapy was missing. What type of issue is this?

- A. Workflow and clinical documentation failure. CORRECT
- B. Pharmacy dispensing issue
- C. Patient preference issue
- D. Completed approval pathway

Explanation: The workflow failed because PA was not checked before scheduling. Documentation also failed because conservative therapy was not included.

Q3. A medication PA says failed prior therapy but gives no dose, dates, duration, or outcome. What should the coordinator do?

- A. Guess the missing information
- B. Submit the same request again
- C. Ask the provider to document complete therapy history. CORRECT
- D. Tell the patient approval is guaranteed

Explanation: Missing therapy history must come from the provider record. The coordinator should not invent facts.

Q4. What can the public PA resource not do?

- A. Support workflow discussion
- B. Explain prior authorization burden
- C. Replace payer specific rules and patient specific benefit verification. CORRECT
- D. Help learners understand documentation risk

Explanation: The resource is educational. It does not replace payer portals, benefit checks, or patient specific review.

Q5. Why is real patient information not used in this activity?

- A. Because fictional examples are harder
- B. Because public learning activities should avoid privacy and compliance risk. CORRECT
- C. Because PA does not involve patients
- D. Because documentation is not important

Explanation: Training should use fictional or deidentified examples only. Real patient identifiers must not be entered into public tools or training activities.

Q6. A PA is approved, but the pharmacy cannot dispense because the approval is linked to another specialty pharmacy. What is the main failure?

- A. Clinical diagnosis failure
- B. Pharmacy routing and post approval access failure. CORRECT
- C. Patient communication preference
- D. Medical necessity failure only

Explanation: The request was approved, but the access pathway was not verified. The team must confirm pharmacy routing, benefit pathway, approval terms, and dispensing status.

Micro Module Recap

The public PA resource is used in Module 1 because learners need a safe way to understand prior authorization before using patient specific tools. It helps explain PA burden, timing, supporting documentation, workflow handoffs, and access delays.

It is linked directly to the MRI scheduling failure, biologic step therapy denial, and oncology pharmacy access case.

The tool supports learning, but it does not replace the provider record, payer rules, benefit verification, pharmacy coordination, or internal workflows.

The learner's job is to convert public resource learning into practical workflow actions, stronger documentation requests, safer handoffs, and more reliable status tracking.

Tool Alignment Summary for Trainers

Tool used

Public PA Resource Workflow

Case study where demonstrated

1. Lumbar MRI scheduled without PA check
2. Biologic medication denied for missing step therapy
3. Approval does not equal access

Why it is synced

The tool is synced to Module 1 because all three cases are foundational workflow problems. Learners are not yet learning how to submit PA requests or write complex appeals. They are learning how to understand burden, timing, documentation gaps, handoff failures, and access delays.

The tool helps learners turn public PA resource learning into practical workflow improvements.